

INTERNAL TRAINING

PARACETAMOL FRESENIUS

10 mg/ml IV infusion

Short-term IV treatment of mild to moderate pain and fever when oral administration is unsuitable.

Dose by weight. Infuse over 15 minutes.

Core adult / paediatric dose bands

>50 kg

1 g 100 ml

Up to 4 times daily

Max: 4 g / 24 h

Interval: ≥4 h

>33 to ≤50 kg

**15
mg/kg**

1.5 ml/kg per dose

Max: 60 mg/kg

and ≤3 g / 24 h

Interval: ≥4 h

>10 to ≤33 kg

**15
mg/kg**

1.5 ml/kg per dose

Max: 60 mg/kg

and ≤2 g / 24 h

Interval: ≥4 h

Renal impairment: if CrCl
≤30 ml/min, leave ≥6 hours
between doses.

Hepatic risk: reduce dose or
prolong interval; max may be
≤2 g/day.

Prepare & give safely

- Use oral route as soon as it is available.
- Single use only: once opened, use immediately and discard unused solution.
- Do not mix with other medicines; may dilute up to one-tenth in 0.9% sodium chloride or 5% glucose.
- After dilution in 0.9% sodium chloride, do not store for more than 1 hour, infusion time included.

**Store at or below 30°C. Do not
refrigerate or freeze.**

Prevent overdose and liver injury.

Red flags before prescribing

Dose using **non-oedematous weight** and include all other medicines containing paracetamol in the daily maximum.

Do not use

Contraindicated in hypersensitivity to paracetamol, pro-paracetamol hydrochloride or excipients, and in severe hepatocellular insufficiency or active liver disease including alcoholic hepatitis.

Unintentional overdose can cause serious liver damage and death.

Monitor during treatment

- Discontinue at first skin rash or hypersensitivity sign; serious skin reactions may be fatal.
- With glass vials, monitor closely to avoid air embolism, especially near end of infusion.
- Warfarin/coumarins: monitor INR during use and for 1 week after discontinuation.
- Flucloxacillin: monitor for high anion gap metabolic acidosis in at-risk patients.